

10/09/2022

Admission Officer
Queensland Royal Hospital

Re: Mrs. Julia Smith, 21 years old
Dear Sir/Madam,

I am writing to refer Mrs. Julia Smith, a 21 year old lady who has recently delivered her baby, for your kind assessment. **Kindly admit the baby in NICU.**

Comment [OET1]: new mother who has delivered in the 30th week of gestation

Please note, Julia was at 30 weeks of her first pregnancy when she delivered her baby. Although she has no previous hospital visits or vaccinations, she used to get monthly visits from her local midwife. Regarding her family history, it is positive for diabetes, hypertension, stroke and inguinal hernia. Please note, Julia was at 30 weeks of her first pregnancy when she delivered her baby. Although she has no previous hospital visits or vaccinations, she used to get monthly visits from her local midwife.

Comment [OET2]: is a visitor from Cyprus who is carrying no medical records

Comment [OET3]: had no ante-natal

On 05/12/2021, Julia presented with severe episodic abdominal and back pain in addition to vaginal bleeding and burst water bag. At 8:04 am, bloody show occurred with ruptured amniotic membranes and strong uterine contractions every two minutes. Her cervical examination revealed six cm dilatation with 90% effacement, as well as station one head engagement. At that time her blood pressure was 140/90, heart rate was 98/min and her baby heart sounds were audible. As a result of being in active labor, delivery of the baby was planned while giving I.V. hydralazine, hydrochloride 5-10 mg every 15-20 minutes to control the blood pressure. At 8:40 am, her blood pressure and heart rate improved to 130/90 and 80/min respectively, and CTG was stable. In addition to this cervical effacement progressed to 100% and head engagement to 4+ with strong frequent uterine contractions. Therefore, hydralazine was stopped. Finally at 9:10 am, baby was delivered with a weight of 4.1 kg. It is worth noting that mother is stable with no vaginal lacerations.

Comment [OET4]: Today morning

Comment [OET5]: Her

Comment [OET6]: had

Regarding Julia's baby, at 9:11 am, his APGAR score was 5, he was not crying and he appeared cyanosed. As a result, massage and back slaps were started which provoked his crying. He was kept on warmer with nasal suctioning. At 9:16 am, his APGAR changed to seven; however, he is currently hypothermic and cyanosed.

Comment [OET7]: ;

Comment [OET8]: and received

Comment [OET9]: improved

Based on the above findings, I believe Julia and her baby need immediate transfer to a hospital with NICU for resuscitation by a pediatrician and management by obstetrician, respectively. Should you have any queries, please do not hesitate to contact me.

Comment [OET10]: to be managed

Comment [OET11]: an

Yours sincerely,
Doctor

Comment [OET12]: faithfully

Report	
Word Length	325
Comments	This is an extremely long letter and still misses some useful information. It makes no effort to write information in brief. Other inaccuracies pertain to word choice and grammar. On the positive side, the purpose is clear. Overall, the letter needs improvements.
Estimated Grade	C+
Estimated Score	310/500
Advice	1. Develop the purpose of the document sufficiently in the introduction. 2. Always read and select case notes carefully. 3. Avoid basic grammar errors. 4. Improve choice of words. 5. Improve coherence/paragraphing. 6. Try to finish the letter in 200 words by writing information in brief wherever possible. Watch this video on word length: https://www.youtube.com/watch?v=AyalEjPv6Ag 7. Always proofread the letter after finishing it.

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